

Kowloon Bay Public Hospital

Discharge Summary

九龍灣公立醫院 出院小結

88 Kwun Tong Road, Kowloon Bay, Kowloon, Hong Kong | Tel: 2790-XXXX | Report No: KB-2026-DS-5521

Name (姓名)	CHEUNG Wai Ming 張偉明	Sex (性別)	Male 男
Age (年齡)	61	Department (就診科室)	Endocrinology 內分泌科
Admission Date (入院日期)	2026-06-12	Discharge Date (出院日期)	2026-06-18

Admission Status (入院情況)

Poorly controlled fasting glucose (14.8 mmol/L, HbA1c 10.2%) and impaired renal function (uACR 185 mg/g) noted on outpatient blood test dated 2026-06-11. Patient also presented with mild bilateral lower limb edema. 血糖控制欠佳 (空腹血糖14.8 mmol/L, 糖化血紅蛋白10.2%), 並見腎功能受損 (尿白蛋白/肌酐比185 mg/g), 雙下肢輕度水腫。

Discharge Diagnosis (出院診斷)

- Type 2 Diabetes Mellitus with Diabetic Nephropathy 2型糖尿病並糖尿病腎病
- Hypertension 高血壓

Hospitalization Process (住院診療經過)

Patient admitted via endocrinology outpatient referral for markedly elevated fasting glucose and albuminuria. IV insulin titration was initiated on admission with close glucose monitoring; subsequently transitioned to oral hypoglycemic therapy (Metformin) once glucose stabilized. Renal function and electrolytes were monitored serially; uACR trended downward on repeat testing. Blood pressure was controlled with ACE inhibitor. No episodes of hypoglycemia or acute complications during admission. 病人經內分泌科門診轉介入院, 因空腹血糖及尿蛋白明顯偏高。入院後即時開始靜脈胰島素滴注並密切監測血糖, 待血糖穩定後轉為口服降糖藥 (二甲雙胍) 治療。住院期間持續監測腎功能及電解質, 覆查尿白蛋白/肌酐比呈下降趨勢。以血管緊張素轉換酶抑制劑控制血壓。住院期間未見低血糖或急性併發症。

Doctor Orders (醫囑)

Continue Metformin 500mg twice daily; low-salt low-sugar diabetic diet; follow up renal function and HbA1c in 3 months at endocrinology clinic; refer to nephrology clinic for co-management. 繼續服用二甲雙胍500毫克, 每日兩次; 低鹽低糖糖尿病飲食; 三個月後於內分泌科門診覆查腎功能及糖化血紅蛋白; 轉介腎科跟進。

Discharge Status (出院情況)

Stable, ambulatory, glucose well controlled on oral medication. Edema resolved. 情況穩定, 可自行活動, 服藥後血糖控制良好, 水腫已消退。